

Sunitinib

Sunitinib is a cancer drug. It is used to treat gastrointestinal stromal tumours (GISTs), kidney cancer and pancreatic neuroendocrine tumours (PNETs).

On this page

- [What is sunitinib?](#)
- [Having sunitinib](#)
- [About side effects](#)
- [Very common side effects](#)
- [Other side effects](#)
- [Other important information](#)
- [How we can help](#)

What is sunitinib?

Sunitinib is used to treat:

- [gastrointestinal stromal tumours \(GISTs\)](#)
- [a type of kidney cancer called renal cell carcinoma](#)
- [pancreatic neuroendocrine tumours \(PNETs\)](#).

Sunitinib is a type of targeted therapy drug called a cancer growth inhibitor. It is also an angiogenesis inhibitor.

It is best to read this information with our general information about [targeted therapy drugs](#) and the [type of cancer](#) you have.

Your cancer team will talk to you about this treatment and its possible side effects before you agree ([consent](#)) to have treatment.

More information about this treatment

This information is correct at time of publishing. But sometimes the types of cancer this treatment is used for, or treatment side effects, may change between revision dates.

You can talk to your cancer team if you want more detailed information about this treatment. Or visit the [electronic Medicines Compendium \(eMC\)](#) website, which has patient information leaflets (PIL) for individual drugs.

Having sunitinib

Sunitinib comes in capsules that you swallow. This means you can take it at home.

During a course of treatment, you will meet someone from your cancer team, such as a:

- cancer doctor
- specialist nurse
- specialist pharmacist.

This is who we mean when we mention doctor, nurse or pharmacist in this information.

During your course of treatment, you will have regular blood tests. This is to check that it is safe for you to have treatment.

You will meet with a doctor, nurse or pharmacist before you have treatment. They will talk to you about your blood results and ask how you have been feeling. If your blood results are okay, the pharmacy team will prepare your targeted therapy.

Your course of treatment

You usually take sunitinib for as long as it is controlling the cancer and any side effects can be managed.

If you have kidney cancer or a GIST, you usually take sunitinib once a day for 4 weeks, followed by 2 weeks without the drug (rest period). This makes up 1 cycle of treatment that lasts for 6 weeks.

If you have a pancreatic neuroendocrine tumour (PNET), you usually take sunitinib once a day every day, with no days off.

Your cancer team may also give you anti-sickness drugs and other medicines to take home. Take these exactly as they tell you to.

Taking sunitinib capsules

Your cancer team will give you the capsules to take home. They may give you capsules of different strengths. Always take the capsules exactly as explained. This is important to make sure they work as well as possible for you.

Make sure you:

- swallow them whole with a glass of water
- do not chew or open them
- take them with or without food
- take them at the same time every day
- do not eat grapefruit or drink grapefruit juice while you are taking sunitinib. Grapefruit can affect how the treatment works and may make side effects worse.

If you forget to take the capsules, take the missed dose as soon as possible within the same day. If a full day has passed, do not take a double dose. Contact the hospital on the 24-hour number you have been given for advice.

Other things to remember about your capsules:

- Keep them in the original package and at room temperature, away from moisture, heat and direct sunlight.
- Keep them safe, where children cannot see or reach them.
- If you are sick (vomit) just after taking them, do not take an extra dose. Take your next dose at the usual time.
- Do not throw away unused capsules. Return them to your cancer team at the hospital.

About side effects

We explain the most common side effects of this treatment here. We also include some that are less common.

You may get some of the side effects we mention, but you are unlikely to get all of them. And you may have some side effects, including rarer ones, that we have not listed here.

Other cancer treatments may cause different side effects. If you are also having other cancer treatment, you may have other side effects.

Always tell your doctor, nurse or pharmacist about any side effects you have. They can give you:

- drugs to help control some side effects
- advice about managing side effects.

It is important to take any drugs exactly as explained. This means they will be more likely to work for you.

Serious and life-threatening side effects

Some cancer treatments can cause serious side effects. Sometimes, these may be life-threatening. Your doctor, nurse or pharmacist can explain the risk of these side effects to you.

Contact the hospital

Your doctor, nurse or pharmacist will give you 24-hour contact numbers for the hospital. If you feel unwell or need advice, you can call at any time of the day or night. Save these numbers in your phone or keep them somewhere safe.

Very common side effects

These side effects happen to 10 or more people in every 100 people (10% or more) who have this treatment.

Risk of infection

This treatment can reduce the number of white blood cells in your blood. These cells fight infection. If your white blood cell count is low, you may be more likely to get an [infection](#). A low white blood cell count is called neutropenia.

An infection can be very serious when the number of white blood cells is low. It is important to get any infection treated as soon as possible. If you have any of the following symptoms, contact the hospital straight away on the 24-hour number:

- a temperature above 37.5°C
- a temperature below 36°C
- you feel unwell, even with a normal temperature
- you have symptoms of an infection.

Symptoms of an infection include:

- feeling shivery and shaking
- a sore throat
- a cough
- breathlessness
- diarrhoea
- needing to pass urine (pee) often, or discomfort when you pass urine.

It is important to follow any specific advice your cancer treatment team gives you.

You will have regular blood tests during treatment. If your white blood cell count is low, your doctor may stop your treatment for a short time, until your cell count increases. Sometimes they will reduce the dose of your treatment.

Bruising and bleeding

This treatment can reduce the number of platelets in your blood. Platelets are cells that help the blood to clot. It can also affect blood vessels.

This means you may bruise or bleed easily. You may have:

- nosebleeds
- bleeding gums
- heavy periods
- blood in your urine (pee) or stools (poo)
- coughing up blood
- tiny red, brown or purple spots that may look like a rash – these spots can be harder to see if you have black or brown skin.

If you have any unexplained bruising or bleeding, contact the hospital straight away on the 24-hour number. You may need a drip to give you extra platelets. This is called a platelet transfusion.

If you have heavy bleeding that happens suddenly or will not stop, call 999.

Anaemia (low number of red blood cells)

This treatment can reduce the number of red blood cells in your blood. Red blood cells carry oxygen around the body. If the number of red blood cells is low, this is called anaemia. You may feel:

- very low in energy
- breathless
- dizzy and light-headed.

If you have these symptoms, contact the hospital straight away on the 24-hour number. You may need treatment for anaemia. If you are very anaemic, you may need a drip to give you extra red blood cells. This is called a blood transfusion.

Feeling tired

[Feeling tired](#) is a common side effect of this treatment. Try to pace yourself and plan your day so you have time to rest. Gentle exercise, like short walks, can help you feel less tired.

If you feel sleepy, do not drive or use machinery.

Constipation

This treatment can cause [constipation](#). Constipation means that you are not able to pass stools (poo) as often as you normally do. It can become difficult or painful. Here are some tips that may help:

- Drink at least 2 litres (3½ pints) of fluids each day.
- [Eat high-fibre foods](#), such as fruit, vegetables and wholemeal bread.
- Do regular gentle exercise, like going for short walks.

If you have constipation, contact the hospital on the 24-hour number for advice. They can give you drugs called laxatives to help.

If you have not been able to pass stools for over 2 days and are being sick, contact the 24-hour number straight away.

Diarrhoea

This treatment may cause [diarrhoea](#). Diarrhoea means passing more stools (poo) than is normal for you, or having watery or loose stools. You may also have stomach cramps. If you have a stoma, it may be more active than usual.

If you are passing loose stools 3 or more times a day and this is not normal for you, contact the hospital as soon as possible on the 24-hour number. Follow the advice they give you about:

- taking anti-diarrhoea medicines
- drinking enough fluids to keep you hydrated and to replace lost salts and minerals
- any changes to your diet that might help.

They might also ask you for a specimen of your stool to check for infection.

Feeling sick

Your doctor, nurse or pharmacist will prescribe anti-sickness drugs to help prevent or control sickness. Take the drugs exactly as they tell you to, even if you do not feel sick. It is easier to prevent sickness than to treat it after it has started.

[If you feel sick](#), take small sips of fluid often and eat small amounts regularly. It is important to drink enough fluids. If you continue to feel sick, or if you are sick (vomit) 1 to 2 times in 24 hours, contact the hospital on the 24-hour number as soon as possible. They will give you advice. They may change your anti-sickness treatment. Let them know if you still feel sick.

Tummy pain

You may get pain in your tummy (abdomen), or have indigestion. Your doctor, nurse or pharmacist can give you advice or treatment to help.

Contact the hospital straight away on the 24-hour number if you get a severe pain in your tummy or if symptoms do not go away, or get worse.

Loss of appetite

This treatment can affect your appetite. Don't worry if you do not eat much for 1 or 2 days. But if your appetite does not come back after a few days, or if you are losing weight, tell your doctor, nurse or pharmacist. They can give you advice. They may give you food or drink supplements. Or they may suggest changes to your diet or eating habits to help.

Changes to your taste

Some foods may taste different or have no taste. Try different foods to find out what tastes best to you. You may also get a bitter or metallic taste in your mouth. Your doctor, nurse or pharmacist can give you advice. It might help to try:

- sucking sugar-free sour or boiled sweets
- eating cold foods
- eating sharp-tasting fresh fruit.

Taste changes usually get better after treatment ends. We have more information about [coping with changes to taste](#).

Sore mouth and throat

This treatment may cause a [sore mouth and throat](#). You may also get mouth ulcers. This can make you more likely to get a mouth or throat infection. Use a soft toothbrush to clean your teeth or dentures in the morning, at night and after meals.

Contact the hospital straight away on the 24-hour number, if:

- a sore mouth or throat affects how much you can drink or eat
- your mouth, tongue, throat or lips have any blisters, ulcers or white patches.

They can give you advice, and mouthwash or medicines to help with the pain or to treat any infection. Follow their advice and make sure you:

- drink plenty of fluids
- avoid alcohol and tobacco
- avoid food or drinks that irritate your mouth and throat.

High blood pressure

This treatment can cause high blood pressure in some people. You will have your blood pressure checked before and during treatment. Or you may be given a blood pressure monitor so you can check it at home. You will be shown how to do this.

Some people may need to take medicine to control their blood pressure. Occasionally, if blood pressure is very high and cannot be controlled, this treatment may be stopped permanently.

If you already have high blood pressure and are having treatment to control it, you will have regular blood pressure checks. Talk to your doctor if you have any concerns.

Skin changes

Sunitinib capsules contain a yellow substance which may make your skin look yellow. If you have black or brown skin, changes in your skin colour may be less noticeable.

You may develop a rash, redness, dryness or itching. If you have black or brown skin the area might become darker.

Your skin may burn more easily during treatment and for several months after. [Use a sun cream](#) of at least SPF 30.

SPF stands for sun protection factor. Cover up with clothing and wear a hat.

Always tell your doctor, nurse or pharmacist about any [skin changes](#). They can give you advice or prescribe creams or medicines to help.

Rarely, this treatment can cause a serious skin reaction that needs to be treated immediately in hospital.

Do not take any more of this treatment and contact the hospital straight away on the 24-hour number if you have any of these symptoms:

- a skin rash that is spreading
- blistering or peeling skin
- flu-like symptoms, such as a high temperature and joint pain
- sores on your lips or in your mouth.

Hand-foot (palmar-plantar) syndrome

This treatment can affect the palms of your hands and the soles of your feet. This is called palmar-plantar or hand-foot syndrome.

If you have white skin these areas may become red. If you have black or brown skin, these areas might get darker.

The skin on the palms of your hands and the soles of your feet may:

-

be sore

- be painful, tingle, or swell
- peel, crack or blister.

If you have any of these symptoms, contact the hospital straight away on the 24-hour number. They can give you advice. This is especially important if you have any broken skin or if walking is difficult. They can prescribe creams and painkillers to help.

You can care for your hands and feet by:

- keeping your hands and feet cool by washing in cool water
- gently moisturising your hands and feet regularly
- wearing gloves to protect your hands and nails when working in the house or garden
- wearing loose cotton socks and avoiding tight-fitting shoes and gloves.

Effects on hair

Your hair may become lighter in colour. Sometimes hair becomes thinner during treatment. If you are worried about this, ask your nurse for advice on hair care.

Feeling dizzy

This treatment may cause dizziness. Tell your doctor or nurse if this is a problem. Do not drive or operate machinery if you are dizzy.

Fluid build-up

This treatment can cause a build-up of fluid in the body. Contact the hospital on the 24-hour number if you:

- are gaining weight
- have swelling in your face, legs or ankles.

They can give you advice and treatment to help.

Thyroid changes

This treatment can affect the thyroid gland. You will have regular blood tests to check how well your thyroid is working during treatment. Possible symptoms of thyroid changes include:

- tiredness
- feeling depressed
- difficulty concentrating
- weight gain
- constipation
- feeling cold
- dry skin

- dry hair.

Tell your doctor if you notice any of these symptoms.

Headaches

This treatment may cause headaches. If you have headaches, tell your doctor, nurse or pharmacist. They can give you advice about painkillers that may help. Tell them if the headache does not get better, or gets worse.

Difficulty sleeping (insomnia)

Sometimes this treatment can affect your sleep. Tell your doctor, nurse or pharmacist if cancer treatment makes it [difficult to sleep](#).

Effects on the lungs

This treatment can cause changes to the lungs. Tell your doctor, nurse or pharmacist if you develop:

- a cough that does not go away
- wheezing
- breathlessness.

You should also tell them if any existing breathing problems get worse. You may have tests to check your lungs.

Back pain or joint and muscle pain

Some people have back pain or joint pain while having treatment. Less commonly, sunitinib can cause muscle pain or cramps. Your doctor can prescribe [painkillers](#) to help with pain.

Other side effects

These side effects happen to less than 10 in 100 people (less than 10%) who have this treatment. Some of them are much rarer than this but they are still important to know about. Rare means a side effect that happens to less than 1 in 1,000 people (less than 0.1%).

Numb or tingling hands or feet (peripheral neuropathy)

This treatment may affect the nerves in your fingers and toes. This can cause numbness, tingling or pain in your hands or feet. This is called peripheral neuropathy. You might find it hard to do fiddly tasks such as fastening buttons or tying shoelaces.

If you have these symptoms, always tell your doctor, nurse or pharmacist.

Nail changes

This treatment can affect [your nails](#). They may grow more slowly or break more easily. Your nails may also turn yellow during treatment. This is because sunitinib contains a yellow-coloured substance.

You might notice ridges or white or dark lines across your nails. These changes usually disappear as the nails grow out after treatment. Sometimes nails can become loose or fall off. Tell your doctor, nurse or pharmacist about any changes to your nails.

If the skin around your nails becomes sore and swollen, contact the hospital straight away on the 24-hour number. These might be signs of an infection.

Blocked or dry nose

This treatment may cause a blocked or dry nose. Tell your doctor, nurse or pharmacist if this is a problem for you. They can give you advice.

Effects on the heart

This treatment can affect how the heart works. You may have [tests](#) to check how well your heart is working. These may be done before, during and after treatment.

If the treatment is causing [heart problems](#), your doctor may change the type of treatment you are having.

Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- breathlessness
- dizziness
- changes to your heartbeat (palpitations)
- swollen feet and ankles.

Other conditions can cause these symptoms, but it is important to get them checked by a doctor.

Always call **999** if you have:

- chest pain, pressure, heaviness, tightness or squeezing across the chest
- difficulty breathing.

Effects on the brain

Rarely, this treatment causes a brain condition that can be serious. You can make a full recovery from this. But it must be diagnosed and treated quickly.

This condition can cause:

- a headache that does not get better
- drowsiness or confusion
- changes in eyesight
- fits (seizures).

If you have any of these symptoms, it is important to either:

- contact the hospital straight away on the 24-hour number
- go to the hospital straight away.

You should not drive yourself to hospital.

Effects on the kidneys

This treatment can affect how the kidneys work. You will have blood tests and may have a urine sample test to check how well your kidneys are working.

Drinking fluids helps protect your kidneys. The advice is usually to try to drink at least 2 litres (3½ pints) of fluid each day. But follow any advice from your doctor, nurse or pharmacist about how much is right for you.

Your urine (pee) may look more yellow during treatment. This is because sunitinib contains a yellow-coloured substance. Contact the hospital on the 24-hour number if you:

- have blood in your urine (pee)
- have foamy, frothy or bubbly urine
- are passing less urine or peeing less often than usual.

Effects on the liver

This treatment may affect how your liver works. This is usually mild and goes back to normal after treatment ends. You will have blood tests to check how well your liver is working.

Blood clot risk

This treatment may cause a [blood clot](#). Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- throbbing pain or swelling in a leg or arm
- reddening of the skin in the area – if you have black or brown skin, this can be harder to notice, but the skin might become darker
- suddenly feeling breathless or coughing

Always call 999 if you have:

- chest pain
- difficulty breathing.

You can help reduce the risk of developing a blood clot by:

- [staying active](#) during treatment
- drinking plenty of fluids, especially water.

You may be given drugs called anticoagulants to help prevent a clot. These thin the blood.

Getting a blood clot is serious, but it can also be treated with anticoagulants. Your doctor, nurse or pharmacist can give you more information about preventing and treating blood clots.

Jaw problems (osteonecrosis)

Rarely, this treatment may cause healthy bone tissue in the jaw to become damaged and die. This is called osteonecrosis of the jaw. It is more likely to affect people who have recently had treatment with a drug used to strengthen the bones ([bisphosphonates](#)).

Dental problems can increase the risk of osteonecrosis. Before you start this treatment, your doctor may advise you to have a dental check-up. It is important to look after your teeth and have regular dental check-ups. Always tell your dentist that you are taking sunitinib.

The symptoms of osteonecrosis of the jaw include:

- pain, swelling or redness of the gums
- loose teeth
- a feeling of numbness or heaviness in your jaw.

Tell your cancer doctor and dentist straight away if you have any of these symptoms.

Tumour lysis syndrome (TLS)

Some people are at risk of developing a condition called tumour lysis syndrome (TLS). The risk is highest at the beginning of treatment. TLS happens when treatment makes large numbers of cancer cells die and break down quickly. This releases lots of waste products into the blood and can affect the kidneys and heart.

TLS can be prevented or treated. You will have regular blood tests to check for TLS. You may have:

- extra fluids through a drip
- medicines such as allopurinol tablets or [rasburicase](#) through a drip.

Drinking at least 2 litres (3½ pints) of fluid a day will also help.

Slow wound healing

Wounds may take longer to heal while you are taking this treatment. If you are having surgery or teeth removed, you may need to stop taking sunitinib before it and for a few weeks afterwards. Your doctor will give you more advice.

Lower blood sugar levels

This treatment may lower blood sugar levels. If you have [diabetes](#), you may need to check your blood sugar levels more often. Your doctor will talk to you about how to manage this.

Hot flushes and sweats

Hot flushes are a common side effect of this treatment. During a flush, your neck and face may feel warm. If you have white skin, your skin may look red. If you have black or brown skin, your skin colour may deepen. Flushes can last up to 5 minutes. You might also have sweats or feel anxious or irritable during a flush.

Here are some things you can do to reduce the effects of hot flushes:

- Wear natural fabrics, such as cotton.

- Wear layers so you can remove clothes as needed.
- Use cotton sheets and layers of bedding you can remove.
- Try cooling pads or pillows to keep you cool.
- Keep rooms cool or use a fan.
- Have cold drinks and avoid caffeine and alcohol.

You might have fewer hot flushes as your body adjusts to this treatment. You could try therapies to help you [cope with hot flushes](#), such as:

- [talking therapies](#) like cognitive behaviour therapy (CBT)
- controlled breathing exercises
- yoga
- acupuncture.

If they do not improve, talk to your doctor. Certain drugs can help to improve hot flushes.

Eyesight changes

This treatment can affect your eyes and eyesight. Your eyes may become watery or sore. Your doctor, nurse or pharmacist can give you advice and eye drops to help with this.

Contact the hospital straight away on the 24-hour number if:

- you have swollen eyes
- you have pain in your eyes
- you notice any changes in your vision.

Other important information

Other medicines

Some medicines can affect how this treatment works or be harmful while you are having it. Always tell your cancer doctor, nurse or pharmacist about any drugs you are taking or planning to take, such as:

- medicines you have been prescribed
- medicines you buy in a shop, pharmacy or online
- vitamins or supplements
- [herbal drugs and complementary or homeopathic therapies](#)
- recreational drugs – for example, cannabis.

Vaccinations

Cancer doctors usually recommend that people with cancer have vaccinations for flu and [coronavirus \(covid\)](#). They may also recommend other vaccines, such as Shingrix[®] for shingles. These all help reduce your risk of serious illness from these infections. Most people can have these vaccines, including people with weak immune systems.

You should not have live vaccines if your immune system is weak. This includes if you are having or recently had chemotherapy, radiotherapy or other cancer treatments that affect your immune system. Live vaccines can make you unwell because they contain a very weak version of the illness they protect you against. There are several live vaccines, including the yellow fever vaccine.

It is important to ask your doctor, nurse or pharmacist for advice about having vaccinations. They can explain what vaccines are right for you and when it is best to have them.

Contraception

Your doctor, nurse or pharmacist will advise you not to [get pregnant](#) or [make someone pregnant](#) while having this treatment and for some time afterwards. The drugs may harm a developing baby. It is important to use contraception to prevent pregnancy. Follow their advice about:

- what types of contraception to use
- how long after treatment you should continue to use contraception.

Breastfeeding

You are advised not to [breastfeed](#) while having this treatment, or for some time after treatment ends. This is because the drugs could be passed to the baby through breast milk.

Your doctor, nurse or pharmacist can give you more information.

Fertility

Some cancer drugs can affect whether you can [get pregnant](#) or [make someone pregnant](#). If you are worried about this, it is important to talk with your doctor before you start treatment.

Medical and dental treatment

If you need medical treatment for any reason other than cancer, always tell the healthcare professional that you are having cancer treatment. Give them the contact details for your cancer doctor or cancer team so they can ask for advice.

If you have appointments with a dentist, always tell them you are having cancer treatment. Talk to your cancer team before you have any dental treatment.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References



Visit the [electronic Medicines Compendium \(eMC\)](#) to download a Patient Information Leaflet (PIL) for more detailed information. The leaflet lists all known side effects.

[Cancer Information Development team](#)

Author

Date reviewed

Reviewed: 01 July 2026 | Next review: 01 July 2028

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

How we can help

How to print a PDF from this website

Instructions on how you can make a PDF version of this page using the print function on your browser.

Clinical Information Nurse Specialists

Our Cancer Information Nurse Specialists are dedicated cancer nurses available to talk to on our Macmillan Cancer Support Line.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

[Get help](#)